

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: RSanchez **Period:** September 17 to October 17, 2024 **Report Date:** October 18, 2024 **Coverage:** Bed: 605/730h (82.9%)
Chair: 522/730h (71.5%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 31 days from Sep 17 to Oct 17



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
43	50h	High Heart Rate	1	100%

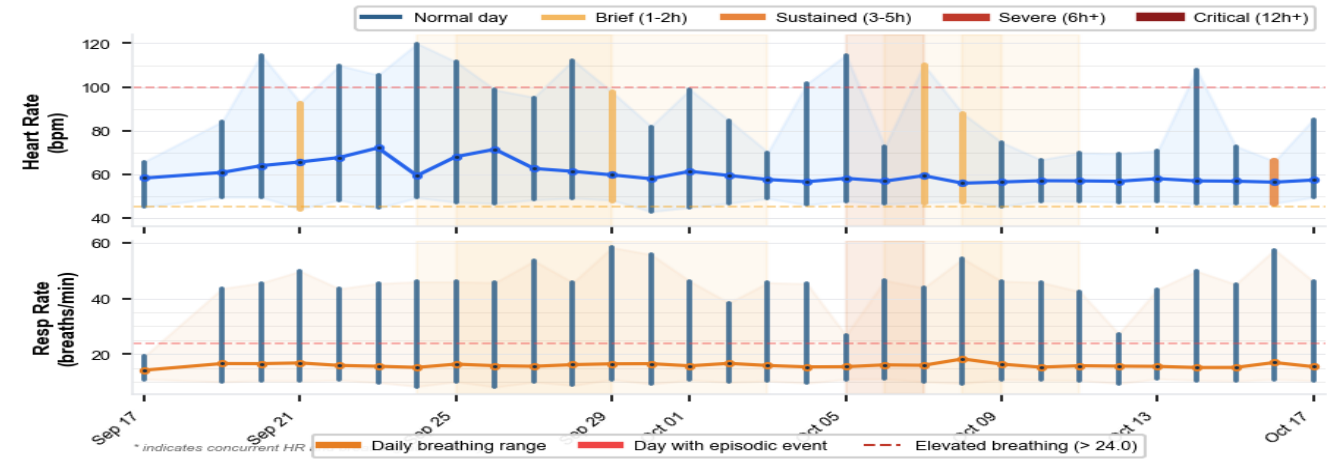
Major Findings: Sustained high HR (avg 58, peak 120)

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	High Heart Rate	115 bpm	3h	2h	<1	58 bpm	Oct 05 to Oct 07
2	Elevated Heart Rate	108 bpm	2h	1h	<1	58 bpm	Oct 05 to Oct 07
3	High Breathing	58 brpm	15h	3h	1	16 brpm	Sep 24 to Oct 03
4	High Breathing	54 brpm	3h	1h	2	17 brpm	Oct 08 to Oct 09
5	Elevated Breathing	54 brpm	9h	2h	1	16 brpm	Oct 06 to Oct 11
6	Elevated Breathing	57 brpm	6h	1h	1	16 brpm	Sep 25 to Sep 29
7	Very High Breathing (brief)	54 brpm	2h	1h	2	50 brpm	Oct 08

The P5 to P95 heart rate spread was 33 bpm (52 to 85), indicating significant cardiac variability.

Clinical Pattern Observations:

- High variability:** HR spread 32 bpm (52 to 85).
- Clustered pattern:** Episodic events on 5 of 31 days (16%).
- Daytime pattern:** Episodes concentrated during waking hours.



Red bars indicate days with detected episodic events.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.